



Dietetics General Outpatients Procedure

1. Scope

Site	Operational Area	Applicable to
Armada Health Service	Dietetics	Dietetics

2. Purpose

The purpose of this procedure is to define the eligibility criteria and process for provision of non-admitted patient dietetic services at Armada Health Service (AHS).

3. Procedure

3.1 Eligibility

- Referrals will only be accepted for patients who reside in the AHS catchment area. Refer to East Metropolitan Health Service (EMHS) [Planning, Innovation & Commissioning HealthPoint](#) page for catchment area.
- Referrals received for patients outside the AHS catchment area will be returned to the referrer or forwarded to relevant health service.
- Referrals from out of catchment areas will only be accepted:
 - if the 'out-of-postcode' patient is seeing a consultant at AHS regularly for follow up and/or
 - if we have a specialised service that no other health service can provide and
 - if we have the capacity to do so without increasing wait list or delay attending to another patient (of similar condition) who is within postcode.
- Must be 18 years or older.
- Referrals for Eating Disorders are not accepted unless the patient has been discharged from AHS with oral nutrition support via the AHS Home Enteral Nutrition (HEN) program. Refer to Armada Kalamunda Group [Registration and Recording Instructions for Home Enteral Nutrition Self Care Procedure](#). These patients will be offered once off follow up whilst waiting for a community based Eating Disorder Treatment and Management Plan.
- Referrals from mental health services (with exception of eating disorders as per 3.1.5) are not accepted.
- Outpatient referrals for patients who dialyse at AHS are not accepted and will be seen in the AHS Dialysis Unit.

3.2 Referrals

- Referral Source: Non-admitted patient single discipline referrals are accepted for AHS patients discharged from medical, surgical and rehabilitation wards, the emergency department and AHS outpatient clinics. Self-referrals and GP referrals will not be accepted unless the patient has been admitted within the last 12 months. Referrals from other hospitals will be accepted as per the dietitian's discretion.

2. Internal referrals are sent via [eReferral](#). External referrals are sent to the Central Referral Service via fax or HealthLink Secure Messaging where they are converted to eReferrals. All eReferrals are [triaged by](#) the Community Rehabilitation triage officer.
3. Community Rehabilitation triage officers accept or reject the referral as per the eligibility criteria for general outpatients. Refer to AKG [Dietetic Paediatric Outpatient Procedure](#) and/or [AKG Adult Diabetes Outpatient Procedure](#) for paediatric and diabetes specific eligibility criteria. Consider referral to Community Rehabilitation service if the patient requires multidisciplinary input.
4. Community Rehabilitation triage officers prioritise referrals as per [Appendix 1 Dietetics Outpatient Clinical Prioritisation Guide](#).
 - Category 1: Urgent – up to 30 days after referral
 - Category 2: Routine – 31-90 days after referral
 - Category 3: Non-urgent – 90-365 days after referral
 - Category 4: Not Accepted – Eligible for receiving services in community
5. Allied health outpatient clerk makes appointment as per the priority time frame and letter sent to patient.
6. Initial appointments are preferably face to face where possible. After the initial appointment, patients are given the option of review face to face or via telehealth (video call or telephone). Refer to [AKG Outpatient Telehealth Policy](#) for details on delivery of telehealth.

3.3 Appointments

1. The clinic is held on a Wednesday and is located in consulting room 2 in Community Rehabilitation and Outpatients.
2. An SMS will automatically be sent 2 days prior to the patient to remind them of their appointment.
3. Dietitian conducts initial appointment and indicate review time frame or discharge on clinic appointment times if attending face-to-face. Response to eReferral is not required.
4. Dietitian to check ISOFT if patient checked in and to call out patients' name at appointment times if attending face-to-face. Dietitian to tick or cross appointment time sheet to indicate attendance or did not attend (DNA).
5. Dietitian to complete service event recording sheet ([Appendix 2](#)) for any additional consults that are not recorded on the appointment time sheet e.g. phone reviews that are not registered, and give to outpatient clerk to add to webPAS.
6. Allied health outpatient clerk registers outcome of appointment and makes follow up appointments as required.
7. Patients who fail to attend 2 scheduled appointments or repeatedly reschedule and/or cancel appointments may be discharged and a discharged letter would be sent to the patient reflecting this. Refer to Health Department of Western Australia (HDWA) [Specialist Outpatient Services Access Policy](#) and AKG [Outpatient Referral Triage and Management Guidelines](#) for details on removing outpatients from the appointment list.
8. New non-admitted patients will be assessed using the [Nutrition Assessment Form AKMR60.9](#). This is to be scanned into the client's digital medical record (DMR) with supporting documentation in corresponding outpatient episode in the DMR indicating to refer to Nutrition Assessment Form AKMR60.9 for the complete assessment. One hour should be allocated to a new non- admitted patient this includes, nutrition assessment, if necessary the provision of oral nutrition support, education and documentation.

9. Follow-up appointments are documented in the outpatient episode in the DMR. A review appointment is based on 30 minutes including contact time, if necessary the provision of oral nutrition support, education, and documentation. The allied health outpatient clerk is responsible for booking follow up appointments.
10. Handover for all outpatients to be completed on iSOFT under HI Nut&DietDept, including date of initial assessment, date last seen, relevant handover details (e.g. anthropometry, HEN) and timeframe for follow up or discharge. Refer to Dietetic Clinical Handover Guideline for instructions on how to complete iSOFT handover, W:\Dietetics & Nutrition\AKH\Dietetics\Policies & Procedures\Department Procedure Manual\Clinical Handover\Dietetics Clinical Handover Guideline.docx.
11. Once therapy has been completed, a discharge letter needs to be completed under clinical letters in DMR and sent to the general practitioner. Responses to internal referrals or other health service site dietetics are not required.

3.4 Data Entry

1. Service events for non-admitted patient data are recorded in webPAS by the allied health outpatient clerk.
2. The dietitian is also required to enter the service event in Allied Health System (AHS) against the corresponding webPAS clinic, DIE 402 Diet Allied Health.

4. Roles and responsibilities

Role	Responsibility
Community Rehabilitation Triage Officer	Receive and triage eReferral
Dietitian	Conduct initial and review appointments Document in patient medical record Inform allied health outpatient clerk of appointment outcome
Allied Health Outpatient Clerk	Books outpatient appointments, new and follow up Requests medical records Records appointment outcome in WebPAS

5. Legislative context

- Health Department of Western Australia (HDWA) [Specialist Outpatient Services Access Policy](#)
- HDWA [Patient Activity Data MP 0164/21](#)

6. Supporting information

- [Registration and Recording Instructions for Home Enteral Nutrition Self Care Procedure AKG-PRO-0389/15](#)
- [Outpatient Telehealth Policy AKG-POL-0532/20](#)
- [Dietetic Paediatric Outpatient Procedure ALL-PRO-0552-20](#)
- [Diabetes Outpatient Procedure AKG-PRO-0563-21](#)
- [Outpatient Referral Triage and Management Guidelines AKG-GUI-0565-21](#)

7. Compliance, monitoring and evaluation

Compliance against this procedure will be evaluated with review of monthly outpatient clinic data including waitlist, attendance numbers and DNA rates.

8. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 5 – Action 5.4: The health service organisation has systems for comprehensive care that:
 - a. Support clinicians to develop, document and communicate comprehensive plans for patients' care and treatment
 - b. Provide care to patients in the setting that best meets their clinical needs
 - c. Ensure timely referral of patients with specialist healthcare needs to relevant services
 - d. Identify, at all times, the clinician with overall accountability for a patient's care

9. Policy Owner

Enquiries relating to this procedure may be directed to:

Title: Coordinator Dietetics and Nutrition
 Department: Dietetics and Nutrition
 Email: ahs.dietetics@health.wa.gov.au

10. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V2.0	02/02/2025	02/02/2028	Scheduled review

11. Authorisation

This procedure has been authorised and issued by the Director Allied Health and Ambulatory Care.

Authorised by	Danielle Kilmurray – Director Allied Health and Ambulatory Care
Authorisation date	27/12/2024
Published date	04/08/2022

Appendix 1 Dietetic Outpatient Clinical Prioritisation Guidelines

Category	First appointment guideline	Examples of relevant scenarios and presenting conditions
1 - Urgent	Immediate Within 24 Hours An immediate outpatient review requires initial telephone contact be made to a specific clinician at the hospital site by the referrer	☐ Chronic kidney disease with blood potassium level >6 mmol/L not on haemodialysis ☐ New diagnosis of anaphylactic food allergy
	Within 30 days	☐ Recent commencement of home enteral feeding ☐ Severely malnourished (as per Subjective Global Assessment (SGA) or at high risk of malnutrition (MST >3) ☐ Lack of appetite &/or nausea & vomiting (if underweight or significant weight loss) or other nutrition impact symptoms ☐ Diarrhoea (if underweight or significant weight loss) or other nutrition impact symptoms ☐ High stoma output (recent GI surgery) ☐ Wounds/pressure injury stage 3 ☐ New diagnosis of anaphylactic food allergy ☐ New medical diagnosis where early nutrition intervention indicated
2 - Semi Urgent	31-90 days	☐ First review post gastrointestinal surgery ☐ Coeliac Disease newly diagnosed ☐ Initial review of HEN prescription ☐ Diverticulitis with significant nutrition impact symptoms ☐ Liver disease, pancreatitis, gallstones ☐ Chronic Kidney Disease (abnormal electrolytes) ☐ Chronic progressive diseases – Parkinson's, Huntington's, Motor Neurone Disease ☐ Poor wound healing (less than stage 3 wound) ☐ Inflammatory bowel disease – recent diagnosis with no previous dietetics input (e.g. Crohns disease, ulcerative colitis) ☐ Heart failure with fluid overload
3 - Not Urgent	90-365 days	☐ Ongoing review of HEN prescription ☐ Other chronic disease requiring periodic review (Diabetes, Chronic kidney disease not on haemodialysis) ☐ Non- anaphylactic food allergy/intolerance ☐ Specific nutritional deficiencies (anaemia)

		☐ Hyperlipidaemia/hypertension/cardiovascular risk patients (<i>who are not suitable for Primary Health Network Groups/Enhanced care program</i>) ☐ Overweight/ obese patients (<i>who are not suitable for Primary Health Network Groups/Enhanced care program</i>) Constipation ☐ Reflux ☐ Arthritis / Gout
4 Not accepted	Referral redirected to primary care	☐ Hyperlipidaemia/hypertension/cardiovascular risk patients (<i>suitable for Primary Health Network Groups/Enhanced care program</i>) ☐ Overweight/ obese patients (<i>suitable for Primary Health Network Groups/Enhanced care program</i>) ☐ Irritable bowel syndrome

Appendix 2 Service Event Recording Sheet

DIETETICS Name of clinician _____

DIETETICS SERVICE EVENT RECORDING SHEET										
Date & Times	UMRN	Patient Name	Service Type NEW, FOL, HMV, TEL, GRP	Clinic Name & Code	Outcome 01) Seen, Make another appt. 03) Seen and discharged from clinic, 12) DNA	Documented in Patient Record	Date of next Appt & Clinic code	Financial Election Public, W/Comp, M/Vehicle, DVA, Overseas, Other	Relevant Comments Referred by if urgent	WebPAS Data Entry Done Date and initial

CODES: DIE400 – Dietetics Paeds DIA401 – Dietetics Diabetes DIE402 – Diet Allied Health

"Service Event" definition:

A non-admitted patient service event is defined as an interaction between one or more healthcare providers with one non admitted patient, which must contain therapeutic/clinical content and result in a dated entry in the patient's medical record.